



STUDY

GLP-1 Drugs: Reshaping Food Category Demand

Analysis of how GLP-1 drugs are reshaping global food category demand, highlighting trends in consumer behavior, market changes, and strategic industry responses.

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Caspr.

STUDY REPORT

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• Executive Summary

Finding 1 — GLP-1s have expanded beyond healthcare into a broad consumer-market force

Between 2024 and 2026, GLP-1 receptor agonists moved from a high-growth therapeutic niche to a major influence on U.S. consumer markets. The impact is extending well beyond healthcare into grocery, foodservice, and consumer packaged goods, driven by high obesity rates and rising adoption of GLP-1 drugs.

Finding 2 — Household penetration is reshaping food purchasing behavior

The expansion of GLP-1 use from 2024 to 2026 materially increased household penetration and changed purchasing patterns, particularly in weight-management and nutrition-focused categories. By 2030, GLP-1 user households are expected to account for a significant share of U.S. food and beverage sales, amplified by concentrated household spending and spillover effects on non-users within the same household.

Finding 3 — Demand is shifting away from indulgence categories toward nutritional utility

GLP-1 adoption is reducing demand for low-nutrient, high-frequency items, with notable pressure on categories such as salty snacks and soft drinks. At the same time, consumers are increasing purchases of nutrient-dense, satiety-oriented products including fresh produce, yogurt, and protein snacks, reflecting prioritization of nutritional utility over calories.

Finding 4 — The change is a reallocation of spend, not simply a reduction in consumption

The evidence suggests consumers are reallocating food spend rather than uniformly cutting demand, with preference moving toward premiumized and functional products. GLP-1 households are also redirecting expenditure to adjacent categories such as fitness, apparel, and self-care, indicating a broader lifestyle-spending shift.

Finding 5 — Adoption appears durable due to medical and reimbursement tailwinds

The report points to sustained GLP-1 adoption as medical indications expand and insurance coverage improves. Usage is especially concentrated among women and middle-aged consumers, making demographic and income mix important determinants of category-level impact.

Finding 6 — Food and beverage portfolios will need structural adaptation

The report frames GLP-1 as a potential structural revenue shift for the food and beverage industry rather than a short-term demand disruption. Recommended responses include reformulating products and launching protein- and fiber-rich offerings, with market strategies anchored in credible nutritional evidence and consumer demand for nutrition efficiency.

1

Introduction and Context

1

Introduction and Context

GLP-1 receptor agonists moved from a high growth therapeutic niche into a broad consumer market force between 2024 and 2026, with implications extending beyond healthcare into grocery, foodservice, and consumer packaged goods. In the U.S., adoption accelerated as obesity prevalence remained structurally high, new indications and products expanded the addressable market, and survey evidence showed a sharp rise in both lifetime and current use, creating a credible pathway for measurable changes in food demand and basket composition.^{11 13 20 15}

This report frames GLP-1 adoption not only as a pharmaceutical story but as a demand reallocation story. The central analytical question is how appetite suppression, altered satiety, and changing food preferences translate into lower demand for some categories and higher demand for others, with consequences for manufacturers, retailers, restaurants, and investors.^{32 28 30}

1.1

Overview of GLP-1 drug adoption trajectory in the U.S. (2024-2026)

The U.S. adoption curve steepened materially from 2024 into 2026. In May 2024, KFF found that 12% of U.S. adults had ever used a GLP-1 drug and 6% were current users; by KFF's 2025 update, 18% reported ever use and 12% reported current use, indicating a doubling in current self-reported use within roughly one year.^{11 13}

- Clinical access expanded the market: FDA approval of Zepbound for chronic weight management in November 2023 added a major new entrant ahead of the 2024 to 2026 acceleration period, complementing Wegovy's earlier obesity positioning and broadening prescriber and patient choice.^{15 23}
- Structural demand conditions remained favorable: U.S. adult obesity prevalence was 40.3% during August 2021 to August 2023, with severe obesity at 9.4%, leaving a very large clinically relevant population even before considering overweight adults with comorbidities.²⁰
- Coverage remained a constraint but not a stopper: KFF reported that only 1% of ACA Marketplace prescription drug plans covered Wegovy for weight loss in 2024, while employer coverage for GLP-1s used primarily for weight loss was still selective, implying that adoption growth occurred despite meaningful reimbursement friction.^{26 21}
- Consumer market relevance became visible by 2025 to 2026: Circana projected that GLP-1 users would represent 35% of U.S. food and beverage sales by 2030, while NIQ reported that GLP-1 households represented 4.5% of households yet drove 43% of sales growth for products with muscle health claims across consecutive 52-week periods in 2024 and 2025.^{28 30}

Taken together, the 2024 to 2026 period should be viewed as the transition from early mass awareness to measurable household penetration. For food sector analysis, the key point is not only how many people use GLP-1 drugs, but that adoption reached a scale where scanner data, survey panels, and category growth patterns began to show statistically and commercially meaningful divergence from non-user households.^{13 29 31}

1.2

Mechanism of appetite suppression and its link to food purchasing behavior

GLP-1 drugs influence food demand because they act on the biological drivers of eating, not just on body weight outcomes. Reviews published in 2024 to 2026 describe appetite reduction as arising from central nervous system signaling that lowers hunger and food reward, alongside peripheral effects such as delayed gastric emptying and enhanced satiety, which together reduce meal size, snacking frequency, and desire for energy-dense foods.^{38 32 39}

- Appetite pathway: GLP-1 receptor agonists reduce appetite and food intake through CNS-mediated effects on satiety and reward processing, helping explain why users often report less interest in highly palatable foods rather than only smaller portions of the same foods.^{38 32}
- Gastrointestinal pathway: Delayed gastric emptying and prolonged fullness can compress eating occasions and increase sensitivity to rich, greasy, or large meals, which is commercially relevant for snacks, desserts, fried foods, and large-format restaurant occasions.^{39 37}
- Behavioral translation: A 2024 narrative review concluded that available studies generally show decreased short-term appetite and food intake under treatment, while retail and shopper datasets in 2025 to 2026 indicate lower overall food and beverage purchasing but stronger orientation toward protein, digestive health, and nutrient-dense products.^{34 29 30}

The link to purchasing behavior is therefore mechanistic as well as observational. If consumers feel full sooner, experience less craving for sugar and fat, and become more intentional about nutrient density, the expected basket effect is lower unit demand in indulgent and convenience categories and relatively better resilience in protein, produce, yogurt, hydration, supplements, and smaller portion formats, a pattern increasingly reflected in industry scanner and survey evidence by 2025 and 2026.^{32 28 31}

1.3

Scope and purpose of the report

This report examines GLP-1 drugs as a cross-sector demand shock affecting food categories, channels, and portfolio strategy. Its purpose is to synthesize clinical evidence on appetite and intake, retail evidence on household purchasing, and market evidence on category winners and losers into a decision-useful framework for industry professionals and investors.^{28 30 13}

- Analytical scope: The report covers adoption trajectory, household penetration, category demand declines, category demand gains, consumer behavior shifts, clinical versus real world evidence, industry response, and long term market implications through 2026, with U.S. data emphasized because it is currently the deepest and most commercially actionable.^{11 20 29}
- Evidence base: The analysis draws on FDA regulatory milestones, CDC and KFF population context, peer-reviewed reviews on appetite and ingestive behavior, and commercial scanner and shopper intelligence from Circana and NIQ to connect biological mechanism with market outcomes.^{15 32 34 28 30}
- Intended use: For manufacturers and retailers, the report identifies where demand erosion or premiumization risk is emerging; for restaurant operators, it highlights portion and menu implications; for investors, it frames GLP-1 exposure as a portfolio allocation and earnings sensitivity issue rather than a narrow healthcare trend.^{28 40}

The report does not assume that all observed food demand changes are caused solely by GLP-1 use. Instead, it evaluates where the evidence is strongest that GLP-1 adoption is amplifying or accelerating pre-existing health, wellness, and premium nutrition trends, and where the magnitude of impact is now large enough to matter for strategic planning and valuation work.²⁹

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2

Adoption Scale and Household Penetration

2

Adoption Scale and Household Penetration

GLP-1 adoption moved from an emerging therapeutic trend to a measurable consumer market force between 2024 and 2026. For this report section, the most decision-useful lens is not only individual prescription growth, but the speed at which use has translated into household penetration, demographic concentration, and a credible pathway to materially larger influence on food and beverage demand by 2030.^{11 13 28}

The evidence base points to three linked conclusions. First, self-reported U.S. adult use rose sharply from 2024 into 2025, with 2026 survey data indicating continued expansion. Second, household penetration is already high enough to affect category-level purchasing patterns, even though user households remain a minority. Third, the addressable population remains very large because obesity and overweight prevalence still far exceed current treatment penetration, implying that adoption can continue rising even under imperfect insurance coverage and affordability constraints.^{74 30 56 62}

2.1

Current U.S. adult usage rates and growth from 2024 to 2026

KFF provides the clearest like-for-like benchmark for all-purpose GLP-1 use across U.S. adults. In May 2024, 12% of adults said they had ever used a GLP-1 drug and 6% said they were current users; by KFF’s November 2025 update, ever use had risen to 18% and current use to 12%, implying a doubling in current self-reported use in roughly 18 months.^{11 13}

Title — U.S. adult GLP-1 adoption benchmarks

METRIC	2024	2025 TO 2026
Ever used GLP-1, all purposes, KFF	12% in May 2024	18% in November 2025
Current GLP-1 use, all purposes, KFF	6% in May 2024	12% in November 2025
Ever used GLP-1 for weight loss, Gallup	6% in 2024	15% in July 2026
Current GLP-1 use for weight loss, Gallup	3% in 2024	11% in July 2026

Source: ¹¹

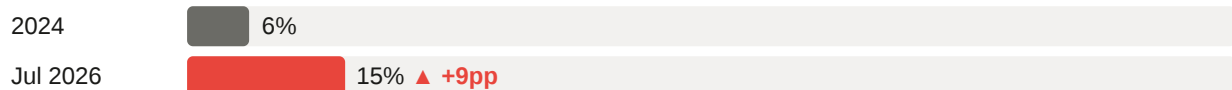
EVER USED GLP-1, ALL PURPOSES (KFF)



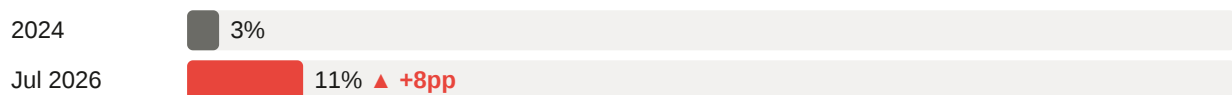
CURRENT GLP-1 USE, ALL PURPOSES (KFF)



EVER USED GLP-1 FOR WEIGHT LOSS (GALLUP)



CURRENT GLP-1 USE FOR WEIGHT LOSS (GALLUP)



■ Earlier period ■ Later period

Gallup's July 7, 2026 survey, while narrower because it focuses on weight loss use rather than all indications, confirms that growth continued into 2026. Gallup found that 15% of U.S. adults had used GLP-1 medications for weight loss at some point and 11% were currently taking them for that purpose, up from 6% and 3% respectively in 2024.⁷⁴

- Growth pattern: The 2024 to 2026 period is best characterized as a transition from early mass awareness to early mass adoption, with current use moving from mid-single digits to low double digits in national surveys.^{13 74}
- Addressable market depth: CDC estimates show 40.3% of U.S. adults had obesity in August 2021 to August 2023, 9.7% had severe obesity, and another 31.7% were overweight, leaving a very large clinically relevant pool relative to current treatment penetration.^{56 20}
- Eligibility versus actual use: Peterson-KFF estimated that more than 2 in 5 adults under age 65 with private insurance could be eligible for GLP-1 drugs under current FDA indications, underscoring how far actual use still sits below theoretical demand.⁶²
- Constraint set: Coverage remains a gating factor rather than a hard ceiling. KFF's 2025 employer survey reported that about 19% of large firms offering health benefits covered GLP-1 drugs for weight loss, up from 18% in 2024, with much higher coverage among the very largest employers.⁷²

For food demand analysis, the key implication is scale. Once current use reaches roughly one in eight adults in broad population polling, the effect is no longer confined to specialty health cohorts and becomes visible in mainstream grocery, mass retail, and restaurant demand data.¹³

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2.2

Household-level penetration and demographic profile of GLP-1 users

Household penetration is commercially more important than individual prevalence because food is purchased at the household level. NIQ reported in its 2026 health and wellness analysis that GLP-1 users represented 4.5% of households in its panel comparison, yet those households drove 43% of sales growth for products carrying muscle health claims across consecutive 52-week periods in 2024 and 2025, showing that even modest penetration can have outsized category effects when spending is concentrated.³⁰

Circana's late 2025 work suggests a much larger household footprint when defined as households containing at least one GLP-1 user. Circana stated in a public 2030 outlook that GLP-1 users are projected to represent 35% of U.S. food and beverage sales by 2030, and a related public presentation cited by search results described GLP-1 users as currently 23% of U.S. households, indicating that household exposure is already materially broader than individual current use rates alone would imply.^{28 79}

- Gender: KFF's 2025 polling shows current GLP-1 use is higher among women than men, consistent with earlier obesity treatment utilization patterns and with the stronger consumer orientation of women in household grocery decision-making.¹³
- Age: Use is concentrated in prime middle-age cohorts rather than the youngest adults, which matters because these cohorts typically control larger household food budgets and have higher rates of obesity, diabetes, and cardiometabolic risk.^{13 20}
- Health status: KFF found much higher current use among adults with diabetes, obesity or overweight, and heart disease than in the general population, confirming that adoption remains clinically anchored even as it broadens into mainstream consumer behavior.¹³
- Socioeconomic profile: Coverage and affordability skew access toward insured and higher-income households, especially those linked to large employer plans. This creates an early adopter profile that overlaps with higher-spending grocery households, magnifying commercial impact per user household.^{72 62}
- Education and income interaction: CDC data show obesity prevalence is lower among adults with a bachelor's degree or more, but the absolute eligible population remains large across all education groups. In practice, higher education and income may reduce clinical prevalence somewhat while increasing treatment access, producing a mixed but commercially attractive user base for premium nutrition categories.²⁰

The market significance of household penetration lies in spillover. A household with one GLP-1 user often changes basket composition for shared meals, snack inventory, and restaurant ordering, so the commercial footprint expands beyond the treated individual. That is why scanner-based household measures can show meaningful category effects even when national individual use rates still appear moderate.^{30 28}

2.3

Projections for adoption through 2030 and their market implications

The most credible 2030 adoption outlooks imply continued expansion rather than saturation. Circana projects that GLP-1 user households will account for 35% of U.S. food and beverage sales by 2030, a forecast that does not require majority household penetration to matter

commercially because user households already spend differently and influence broader household purchasing patterns.²⁸

Several structural drivers support further adoption through 2030. The eligible population remains far larger than the treated population, employer coverage is gradually widening, and NIQ notes that additional regulatory approvals in conditions such as chronic kidney disease, liver disease, heart failure, and sleep apnea could materially expand the addressable market beyond obesity and diabetes alone.^{62 72 30}

- Base case trajectory: If current use remains in the low double digits by 2026 and coverage broadens incrementally, adoption can plausibly move from an early adopter phase to a mainstream chronic care category by 2030. This is an inference from current survey growth, eligibility depth, and expanding payer participation rather than a single published population forecast.^{13 74 62}
- Upside scenario: Faster uptake would likely come from lower net prices, broader employer and public coverage, and label expansion into additional chronic conditions. Under that scenario, food demand effects would diffuse more quickly from premium and health-oriented categories into mass-market center store and foodservice.^{72 30}
- Downside scenario: High discontinuation, reimbursement pullbacks, or persistent affordability barriers could slow the pace of penetration. Even then, the installed base may remain large enough to sustain category-level effects because the highest-spending households are disproportionately represented among early adopters.^{73 30}
- Market implication for manufacturers: Portfolio exposure should be assessed by household penetration sensitivity, not just by total category size. Categories dependent on impulse calories, large portions, or frequent snacking face the greatest long-run risk, while protein-dense, nutrient-efficient, and portion-controlled formats gain strategic relevance as penetration rises.^{28 30}
- Market implication for investors: By 2030, GLP-1 exposure is likely to be a recurring screen in food and beverage valuation work, similar to how investors already assess exposure to private label, channel mix, and health and wellness trends. The importance comes less from total user count alone than from the concentration of spending power and the persistence of basket reshaping at the household level.^{28 74}

In strategic terms, 2030 is close enough that food and beverage companies should treat GLP-1 adoption as a planning variable, not a speculative tail risk. The combination of rising adult use, widening household penetration, and a still underpenetrated eligible population supports a view that GLP-1-driven demand reallocation will remain a live market issue throughout the rest of the decade.^{56 28 30}

3

Food Category Demand Shifts — Losers

3

Food Category Demand Shifts — Losers

The clearest near-term demand pressure from GLP-1 adoption is concentrated in categories built on appetite frequency, indulgence, and large portions. Across scanner-based retail work, shopper surveys, and industry analyses published through 2026, the most exposed categories are salty snacks, sweet baked goods, cookies, carbonated soft drinks, selected frozen and center-store staples, and several quick-service restaurant occasions where value has historically depended on combo meals, add-ons, and impulse consumption.^{30 99 107}

The pattern is not a uniform collapse in food spending, but a selective erosion of low-nutrient, high-frequency categories as users report fewer eating occasions, smaller baskets, and more intentional choices. NIQ reported that shopping trips among GLP-1 users focused on weight loss declined 6.4% year over year, while Circana and other industry sources describe a reallocation away from indulgent center-store products and toward protein, hydration, and function-led items, implying that category losers are primarily those least aligned with satiety efficiency and nutrient density.^{30 101}

3.1

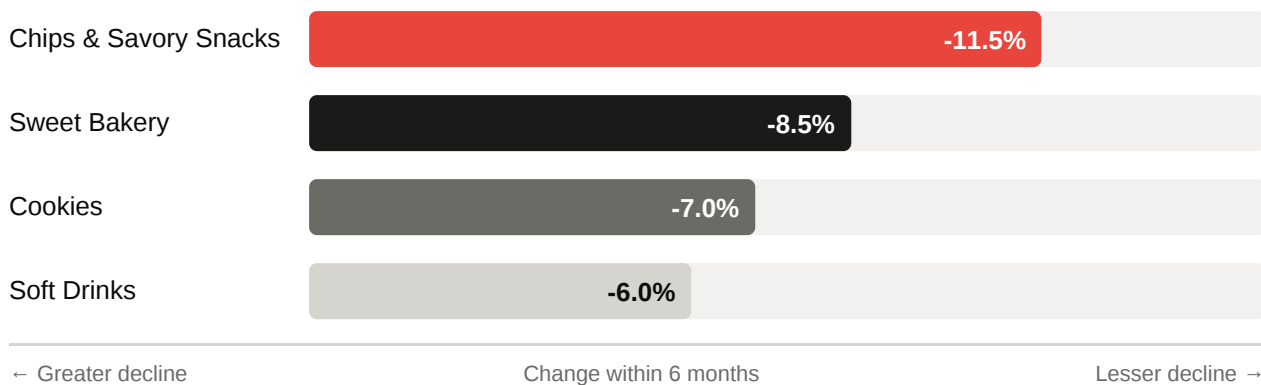
Declines in salty snacks, sweet bakery, cookies, and soft drinks

Retail evidence is strongest in the snack and sweets complex, where GLP-1 use directly reduces grazing behavior and weakens demand for calorie-dense impulse products. OCC Strategy, citing six-month post-adoption grocery spending changes for weight-loss users in 2025, reported declines of 11.5% for chips and savory snacks, 8.5% for sweet bakery, 7% for cookies, and 6% for soft drinks, making this one of the most clearly quantified category-loss clusters in the current evidence base.¹⁰⁷

Title — Grocery category declines after GLP-1 adoption

CATEGORY	CHANGE WITHIN 6 MONTHS	INTERPRETATION
Chips and savory snacks	-11.5%	Highest exposure to reduced snacking frequency.
Sweet bakery	-8.5%	Weakened demand for indulgent baked treats.
Cookies	-7.0%	Lower impulse and dessert purchasing.
Soft drinks	-6.0%	Fewer accompaniment and craving occasions.

Source: ¹⁰⁷



- Salty snacks: This category is especially vulnerable because it depends on habitual between-meal consumption, a behavior NIQ says is being disrupted as GLP-1 users eat fewer main meals and become more deliberate about what earns a place in the basket.^{100 107}
- Sweet bakery and cookies: These categories face a double headwind from lower appetite and lower tolerance for rich foods, with NIQ describing a consumer mindset that favors fewer occasions and higher expectations per occasion rather than routine treat consumption.^{104 100}
- Soft drinks: Carbonated and sugary beverages lose relevance when meals shrink and snack pairings decline, and industry commentary increasingly frames protein beverages, hydration, and functional drinks as taking share from traditional refreshment occasions.^{99 30}

A notable nuance is that the decline is more severe in frequency-led mass categories than in premium indulgence. NIQ's 2026 commentary suggests some users still buy treats, but do so less often and with more selectivity, which implies that mainstream cookies, bakery snacks, and soft drinks are more exposed than occasional premium desserts or specialty beverages.¹⁰⁰

3.2

Impact on frozen foods, cheese, salad dressings, and center-store staples

The pressure extends beyond obvious indulgence categories into center-store staples that are tied to convenience meals, heavy meal assembly, and lower nutrient density. OCC Strategy's 2025 category analysis identified declines in frozen foods, cheese, and salad dressings among GLP-1 households after adoption, indicating that the demand reset reaches into pantry and refrigerated staples, not just discretionary snacking.¹⁰⁷

- Frozen foods: The category is bifurcating rather than uniformly declining. Legacy frozen items associated with comfort food, pizza, and high-calorie convenience are under pressure, while manufacturers have responded by launching portion-controlled, protein-forward frozen meals specifically designed for GLP-1 users, implying that the category loss is concentrated in traditional formats rather than the entire frozen aisle.^{110 97}
- Cheese and salad dressings: These products are exposed because they often function as calorie-dense accompaniments rather than primary nutrition anchors. As users simplify meals and prioritize protein efficiency, add-on categories such as shredded cheese, creamy dressings, and rich toppings can lose units even when core meal consumption remains intact. This is an inference supported by reported category declines and by NIQ's evidence of more intentional, lower-carbohydrate, smaller-meal behavior.^{107 30}
- Center-store staples: Categories built around stock-up behavior, processed meal kits, and routine pantry replenishment are vulnerable when shopping trips fall. NIQ's reported 6.4% year-

over-year decline in shopping trips among GLP-1 users suggests fewer opportunities to add center-store impulse items, while Circana's framing of "evolving baskets" indicates that basket composition is being rebuilt around function rather than shelf-stable abundance.^{30 101}

The market response has been adaptation rather than retreat. Frozen and center-store manufacturers are increasingly repositioning around portion control, protein density, digestive tolerance, and simpler ingredient decks, which itself is evidence that incumbent formats in these aisles are losing relevance with a growing GLP-1 user base.^{110 104}

3.3

Quick-service restaurant category declines (pizza, chicken, burgers, coffee/bakery)

Quick-service restaurants face a structurally different risk from retail because GLP-1 users do not need to eliminate restaurant visits for category economics to weaken. The more immediate effect is lower ticket composition, fewer sides and beverages, reduced dessert attachment, and weaker demand in categories built around large portions and craveability. This is consistent with restaurant industry adaptation in 2026 toward lighter, protein-forward, lower-carbohydrate menu options.^{98 102}

- **Pizza:** Pizza is among the most exposed QSR and delivery categories because it combines large portions, refined carbohydrates, cheese load, and group-ordering dynamics. Industry figures summarized in 2026 market commentary indicate pizza has seen one of the steepest household spending pullbacks among GLP-1 users, reflecting both lower personal intake and reduced appeal of leftovers and multi-slice occasions.^{103 107}
- **Chicken and burgers:** These categories are not disappearing, but they are being reformatted. Operators are promoting bunless burgers, grilled chicken, smaller portions, and protein cups because traditional fried sandwiches, combo meals, and add-on fries are less aligned with GLP-1 eating patterns. That implies category pressure is greatest in legacy high-calorie builds rather than in lean-protein cores.^{98 105}
- **Coffee and bakery:** Coffee-led chains are exposed through pastry attachment, sweetened beverages, and snack occasions rather than coffee alone. As appetite suppression reduces grazing and dessert demand, bakery mix and indulgent drink customization become more vulnerable than plain coffee or high-protein beverage formats.^{103 104}

The key investor takeaway is that GLP-1 pressure on QSR is category-specific and mix-driven, not simply traffic-driven. Even where restaurant frequency remains resilient, the economics of pizza, fried chicken, burgers with sides, and coffee-plus-bakery occasions can deteriorate if users trade down in portion size, skip beverages, or substitute toward protein-led items with lower attachment rates.^{98 102 30}

4

Food Category Demand Shifts — Winners

4

Food Category Demand Shifts — Winners

The demand upside from GLP-1 adoption is concentrated in categories that deliver satiety, nutrient density, digestive tolerance, and health signaling per eating occasion. Across 2025 to 2026 scanner analyses, shopper panels, and industry surveys, the strongest beneficiaries are fresh produce, yogurt, nutrition bars, protein snacks, vitamins, supplements, and adjacent health-oriented products, indicating that GLP-1 is reallocating spend rather than simply shrinking food demand overall.^{30 99 129}

This shift is consistent with the biological and behavioral profile of GLP-1 users already established in earlier sections, but the commercial significance here is category mix. Winning categories are those that help consumers eat less while feeling nutritionally covered, which supports premiumization in protein and wellness while improving the relative resilience of perimeter and function-led categories.^{100 122 101}

4.1

Growth in fresh produce, yogurt, nutrition bars, and protein snacks

The strongest food-category winners are products that fit smaller, more intentional eating occasions while still delivering fullness, protein, hydration, or digestive comfort. McKinsey’s 2026 food and beverage report, citing Cornell SC Johnson research on grocery spending six months after GLP-1 adoption, identifies fresh produce, yogurt, and nutrition bars among the categories showing average spending gains after medication uptake, directly contrasting with declines in indulgent snack categories covered earlier.¹²⁹

Title — GLP 1 linked growth categories

CATEGORY	DIRECTION AFTER ADOPTION	WHY IT IS BENEFITING
Fresh produce	Up	Supports lighter meals, freshness, and nutrient density.
Yogurt	Up	Fits small portions, protein needs, and gut health positioning.
Nutrition bars	Up	Portable, portion-controlled, and functional.
Protein snacks	Up	Align with satiety and muscle maintenance priorities.

Source: ¹²⁹

- Fresh produce: NIQ reported in 2026 that wearable usage is associated with increased water, protein, and fresh produce consumption, and it positioned GLP-1 users within a broader self-directed health consumer shift that is moving spend toward minimally processed, health-signaling foods. This matters because produce benefits not only from calorie reduction, but from substitution away from heavy center-store meal components.³⁰
- Yogurt: NIQ’s 2026 intentional appetite analysis states that newer GLP-1 users over-index in nutritious categories including yogurt and fresh fruit, suggesting yogurt is functioning as a high-protein, portion-friendly, easy-to-tolerate format early in the treatment journey. Expo West 2026

coverage from NIQ also highlighted yogurt brands leaning into probiotic and prebiotic claims, reinforcing yogurt's role at the intersection of protein and gut health.^{100 124}

- Nutrition bars and protein snacks: Circana's 2025 update on GLP-1 households describes a noticeable shift toward snacks offering protein, hydration, energy boosts, and convenience, while NIQ's 2026 health and wellness report says category convergence is accelerating at the intersection of supplements, snacks, and beverages. That combination favors bars, jerky, high-protein bites, and other compact formats that justify their place in fewer eating occasions.^{99 30}

The analytical point is that these categories are not merely "healthy" in a generic sense. They are winning because they solve the GLP-1 consumption equation of lower appetite, higher selectivity, and greater demand for nutritional efficiency per bite, which gives perimeter fresh and functional snack formats a structural advantage over frequency-led indulgence categories.^{104 101}

4.2

Increased spending on vitamins, supplements, and health-oriented products

GLP-1 adoption is also redirecting spending into non-food wellness categories that consumers perceive as supporting metabolic health, nutrient adequacy, digestive comfort, and overall self-management. NIQ's 2025 Consumer Outlook to 2026 explicitly identifies incrementality in CPG for fiber and protein, digestive health, supplements and vitamins, and notes that consumers are shifting spend away from food excess toward medications, lifestyle, and health-oriented categories.¹²³

- Supplements and vitamins: NIQ's 2026 health and wellness analysis says winning categories are emerging where supplements, snacks, and beverages converge, indicating that consumers increasingly want function embedded into everyday consumption rather than separated into a purely pharmaceutical routine. This is especially relevant for GLP-1 users managing reduced intake and seeking reassurance around nutrient sufficiency.³⁰
- Health-oriented basket expansion: Circana's GLP-1 consumer behavior materials describe evolving baskets rather than shrinking appetites, with stronger orientation toward protein, hydration, energy support, and convenience. That framing implies that vitamins and wellness products are part of a broader basket redesign around intentional health maintenance, not an isolated add-on purchase.^{125 101}
- Market signaling: NIQ's Expo West 2026 review described an expanded universe of metabolic-supporting supplements tailored to more health-attentive consumers, showing that manufacturers and retailers are already treating GLP-1-influenced demand as a product development and merchandising opportunity. The commercial implication is that supplement growth is being reinforced by both consumer pull and supply-side innovation.¹²⁴

Purchasing behavior appears to be changing in two ways. First, consumers are reallocating dollars from discretionary calories toward products that signal control, adequacy, and wellness. Second, they are increasingly comfortable buying across category boundaries, such as protein-plus-gut-health or snack-plus-supplement formats, which expands the addressable market for health-oriented products beyond the traditional vitamin aisle.^{123 30}

This matters for investors because supplements and adjacent wellness products can capture a disproportionate share of wallet even if total food volume falls. In a GLP-1 environment, categories that help consumers feel nutritionally optimized may benefit from both higher

frequency of consideration and stronger premium acceptance than conventional packaged foods.^{100 123}

4.3

Shifts in willingness to pay and price sensitivity for protein products

Protein is the clearest example of GLP-1-driven premium resilience. A 2025 Food Policy study found that GLP-1 use increases willingness to pay for most evaluated protein products and reduces price sensitivity for some protein goods, meaning users' purchase volumes respond less to price changes than those of non-users.^{122 127}

- Lower price sensitivity: The Food Policy findings imply that discounting is less effective as a demand lever for GLP-1 protein shoppers than for conventional shoppers. For manufacturers, that supports a strategy centered on value-added protein formats, quality cues, and functional differentiation rather than pure price competition.¹²²
- Higher willingness to pay: NIQ's 2025 Consumer Outlook reported that more than half of consumers say they will pay a premium for fresh, clean, and healthier options, while NIQ's 2026 intentional appetite analysis recommends sharper pack-price architecture and clearer value communication for GLP-1-influenced shoppers. Together, these sources suggest that protein products can sustain premium pricing when the benefit is explicit, credible, and easy to integrate into smaller eating occasions.^{123 100}
- Buyer mix effect: The AgEconSearch version of the protein-demand study notes that lower own-price sensitivity implies GLP-1 users may represent a larger share of aggregate protein purchases as prices rise, because non-users cut back more sharply. That creates a subtle but important mix shift in which protein categories become increasingly dependent on a consumer segment that values satiety and function over lowest price.¹²⁷

The broader implication is that GLP-1 is not only changing what consumers buy, but how they evaluate value. In protein, value is moving away from calories per dollar and toward satiety, nutrient density, convenience, and tolerance, which supports premium snacks, dairy protein, meat snacks, and fortified formats even in a generally price-conscious consumer environment.^{99 124}

For category strategy, this means protein should be treated as a margin opportunity as much as a volume opportunity. Brands that can demonstrate clear functional payoff, portion appropriateness, and clean-label credibility are better positioned than brands relying on commodity protein claims alone.^{122 100}

5

Consumer Behavior and Lifestyle Transformation

5

Consumer Behavior and Lifestyle Transformation

GLP-1 adoption is increasingly associated with a broader lifestyle reset rather than a narrow reduction in food volume. Clinical evidence through 2026 shows meaningful reductions in energy intake under treatment, while commercial panel and survey data indicate that users often translate lower appetite into more intentional eating, fewer processed-food occasions, and higher spending on categories linked to health management, body change, and self-presentation.^{142 139 30}

The long-term significance for industry is that these changes appear to diffuse beyond the treated individual and into household routines, wardrobe needs, exercise behavior, and self-care purchases. At the same time, discontinuation remains a major fault line: available 2025 to 2026 reviews suggest that biological rebound after stopping therapy is common, so the durability of lifestyle transformation depends less on the drug alone than on whether new habits become embedded in household systems and consumer identity.^{140 153 165}

5.1

Caloric reduction patterns and substitution toward minimally processed foods

Clinical evidence now supports a sizable reduction in energy intake during GLP-1 therapy. A 2026 systematic review and meta-analysis of quantitative dietary outcomes found pooled energy intake reductions of about 1,132 kJ in the core analysis, rising to about 1,421 kJ when an additional phase 1 trial was included, equivalent to roughly 270 to 340 kcal per day.¹⁴²

Title — GLP 1 linked intake and food pattern shifts

INDICATOR	REPORTED CHANGE	COMMERCIAL IMPLICATION
Energy intake reduction, core meta analysis	- 1,132 kJ/day	Lower total calorie demand under treatment.
Energy intake reduction, expanded analysis	- 1,421 kJ/day	Stronger downside risk for frequency-led food categories.
Grocery spending after adoption	- 5.3% within 6 months	Calorie reduction translates into smaller food baskets.
Grocery spending in user households	About -6% within 6 months	Household-level demand effects emerge quickly.

Source: ^{142 158 149}



Energy bars scaled to max 1,421 kJ/day | Spending bars scaled to max 6.0%

The dietary shift is not only quantitative but compositional. BCG reported in 2026 that non-users living with a GLP-1 user also tend to eat fewer processed foods, more fruits and vegetables, and less fast food, while NIQ's 2026 health and wellness analysis linked GLP-1 households with stronger orientation toward fresh produce, protein, and other function-led foods.^{140 30}

- Mechanism to basket translation: Lower appetite and greater satiety reduce the value of calorie-dense convenience foods, making minimally processed foods more competitive when consumers are choosing fewer eating occasions.^{142 145}
- Nutritional efficiency effect: When consumers eat less overall, they appear to prioritize foods that deliver protein, fiber, hydration, and micronutrient value per serving, which favors produce, yogurt, lean proteins, and simpler meal formats over highly processed snacks.^{30 144}
- Investor implication: The relevant risk is not just fewer calories sold, but a quality mix shift away from ultra-processed, frequency-dependent categories and toward foods perceived as cleaner, lighter, and more purposeful.^{145 158}

5.2

Household spillover effect and behavioral persistence after discontinuation

Household spillover is now one of the most commercially important features of GLP-1 adoption. PwC reported that as of May 2026, about 21% of U.S. households included a current GLP-1 user, up from 9% in January 2025 based on Numerator analysis, while BCG found that non-users living with a GLP-1 user adopt roughly 30% to 50% of the user's behavior changes, including eating fewer processed foods and less fast food and consuming more fruits and vegetables.^{139 140}

This spillover helps explain why household-level demand effects can exceed what individual prevalence alone would suggest. The 2025 consumer demand study by Hristakeva, Liaukonytė, and Feler estimated an average grocery spending decline of 5.3% in the six months after adoption, and Numerator-linked public slides reported an approximately 6% decline for households with at least one GLP-1 user, indicating that shared pantry, meal planning, and restaurant decisions amplify the effect.^{158 149}

Persistence after discontinuation is more uncertain and should be treated cautiously. Reviews published in 2026 consistently conclude that stopping GLP-1 therapy is commonly followed by

weight regain and deterioration in cardiometabolic markers, with the BMJ meta-analysis estimating average regain of 6.0 kg within the first year after stopping incretin mimetics and a projected return to baseline weight around 1.6 years after cessation.^{165 153}

- Biological persistence is weak: Current evidence suggests the pharmacologic appetite effect fades after discontinuation, so behavioral persistence cannot be assumed from treatment history alone.^{153 152}
- Behavioral persistence is conditional: Reviews on post-discontinuation maintenance emphasize that sustained outcomes depend on integrated lifestyle support, structured nutrition habits, and ongoing behavioral reinforcement rather than medication withdrawal by itself.^{156 162}
- Commercial implication: Even if some individual users discontinue, household routines learned during treatment may preserve part of the basket shift, especially where the treated user changed shared shopping lists, meal composition, and restaurant norms. This is an inference from the coexistence of strong household spillover evidence and weak biological persistence after cessation.^{140 165}

5.3

Spending reallocation toward fitness, apparel, and self-care

The consumer wallet effect increasingly extends beyond food. PwC reported in 2026 that 73% of current GLP-1 users reported a meaningful change in clothing size, 26% said they were spending more on clothing overall, and apparel spend was up 9.9% among GLP-1 users after 6 to 8 months on the drug.¹³⁹

Fitness and health-management spending are also rising, although unevenly by user segment. PwC found that 36% of current users had increased cardio activity such as walking, running, cycling, or swimming, while BCG's 2026 segmentation described a large "lifestyle optimizer" cohort, representing 41% of surveyed users, that showed the strongest spending increases in fitness memberships, wearables, and apparel.^{139 140}

Self-care categories are benefiting as body change and health attention spill into adjacent purchases. NIQ reported that GLP-1 weight-loss users spend 1.3 times more on health and beauty products than nonusers, including 97% more on fragrance, 58% more on sun care, 53% more on hair products, and 39% more on deodorants.³⁰

- Apparel reactivation: Weight loss creates both functional replacement demand and identity-driven spending, with users buying smaller sizes, activewear, and transitional wardrobes as their bodies change.^{146 147}
- Fitness reallocation: Increased exercise is not universal, but a meaningful share of users appear to complement medication with cardio, wearables, and structured wellness routines, supporting adjacent demand in gyms, connected fitness, and sports apparel.^{139 140}
- Self-care premiumization: Beauty, grooming, and body-confidence categories are emerging as secondary beneficiaries of GLP-1 adoption, suggesting that the economic effect is partly a reallocation from calories toward appearance, maintenance, and health signaling.^{30 139}

For investors, this means GLP-1 exposure should be assessed across a wider consumer ecosystem than food alone. The strongest medium-term beneficiaries may include apparel, activewear, wearables, beauty, and wellness services that monetize sustained body change and health engagement, even if food volume growth slows.^{140 30 139}

6

Clinical and Retail Evidence Review

6

Clinical and Retail Evidence Review

This section connects two evidence streams that are often discussed separately. Clinical studies measure how GLP-1 and dual incretin therapies change energy intake under standardized conditions, while retail and household panel studies observe how those physiological effects translate into grocery and foodservice spending in everyday life.^{142 189}

The integration challenge is that these literatures use different units, time horizons, and populations. Clinical meta-analyses typically report kilojoule or kilocalorie reductions in controlled settings, whereas scanner studies report changes in dollars, trips, and category mix, making translation from intake suppression to category demand neither automatic nor one to one.^{142 190}

6.1

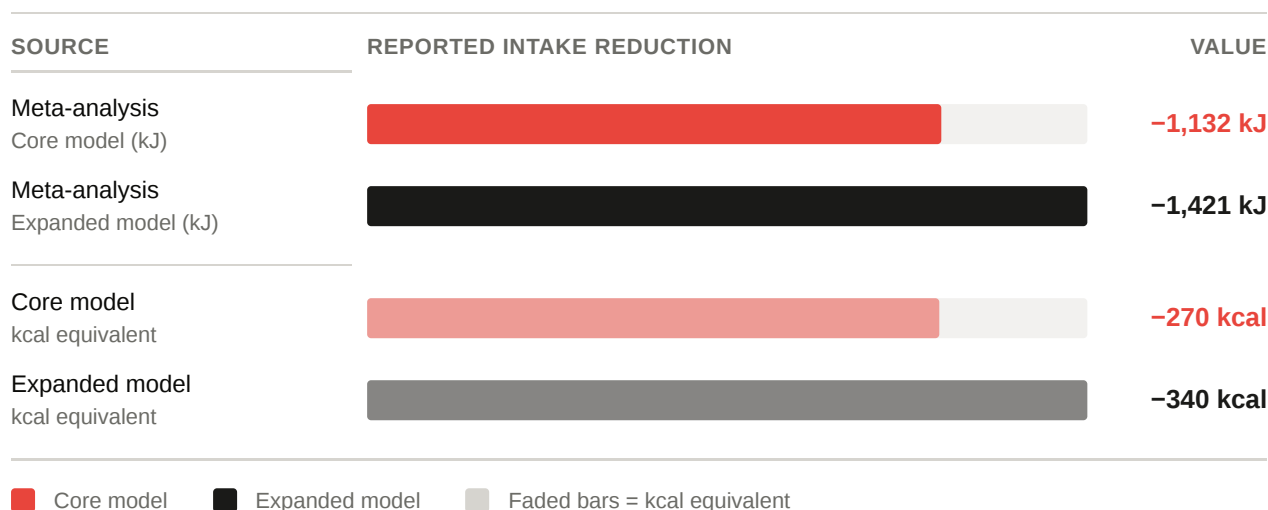
Meta-analytic findings on caloric intake reduction under standardized conditions

The strongest current synthesis is the 2026 systematic review and meta-analysis of quantitative dietary outcomes during GLP-1 or dual GIP GLP-1 therapy. It searched six databases through March 31, 2026 and found pooled reductions in energy intake of about 1,132 kJ per day in the core analysis, rising to about 1,421 kJ per day when an additional phase 1 trial was included, equivalent to roughly 270 to 340 kcal per day under study conditions.¹⁴²

Title — Standardized intake reduction evidence

EVIDENCE SOURCE	REPORTED INTAKE EFFECT	INTERPRETATION
2026 dietary meta analysis core model	-1,132 kJ/day	Meaningful reduction in spontaneous energy intake under treatment.
2026 dietary meta analysis expanded model	-1,421 kJ/day	Upper bound estimate when additional phase 1 evidence is included.
Approximate kcal equivalent of core model	-270 kcal/day	Commercially relevant but not directly translatable to spend.
Approximate kcal equivalent of expanded model	-340 kcal/day	Suggests stronger downside risk for calorie dense categories.

Source: ¹⁴²



- Standardization strength: These estimates are valuable because they aggregate studies using structured dietary assessment and controlled treatment contexts, reducing noise from seasonality, household sharing, and self-directed diet changes.¹⁴²
- Mechanistic consistency: The 2025 scoping review on nutrition strategies for semaglutide and tirzepatide concluded that the evidence base consistently points to reduced appetite, lower spontaneous intake, and the need for nutrition support to preserve protein adequacy and diet quality during treatment.^{170 199}
- Boundary condition: Even this meta-analytic evidence should not be read as a direct forecast of category sales loss, because standardized meal tests and trial-linked dietary records capture biological response more cleanly than they capture substitution across foods, channels, and household members.^{142 172}

A useful nuance for investors is that the clinical literature is stronger on total intake suppression than on dietary quality improvement. The CRAVE study, published in 2026, reported reductions in energy intake and micronutrient consumption in a real-world cohort without structured nutrition support, but no improvement in diet quality or food cravings, reinforcing that lower calories do not automatically imply better habitual eating patterns.¹⁷²

6.2

U.S. scanner data and household expenditure analysis

The most decision-useful U.S. retail evidence comes from linked household panel and transaction data rather than from self-reported food diaries. The Journal of Marketing Research paper by Hristakeva, Liaukonytė, and Feler uses survey responses on GLP-1 adoption linked to a representative U.S. household transaction panel from July 2022 to September 2024 and finds that households with at least one GLP-1 user reduce grocery spending by 5.3% within six months of adoption, with the decline reaching about 8.2% among higher-income households.¹⁸⁹

- Household level effect: The same study is important because it measures spending at the household level, which is the relevant unit for grocery demand, pantry stocking, and shared meal decisions rather than only the treated individual's intake.¹⁸⁹

- Channel extension: Public summaries of the research report an approximately 8% to 8.6% decline in spending at fast-food chains, coffee shops, and limited-service restaurants, indicating that the effect is not confined to at-home grocery baskets.^{188 196}
- Category mix insight: Numerator’s 2026 public analysis shows GLP-1 households decreased food spending by 1.6% in the first nine months of the year versus the prior year, compared with a 0.5% decline for non-GLP-1 households, while packaged sweet bakery goods, chips, packaged cookies, and cold cuts underperformed in GLP-1 households and items such as sugar substitutes, canned fruit, and dried fruit performed relatively better.¹⁹⁰
- External validation: McKinsey’s 2026 food and beverage report reproduces the broad pattern from the Cornell linked-panel work, showing average spending declines in indulgence-heavy categories and relative resilience or gains in produce, yogurt, and nutrition bars after adoption.¹²⁹

For industry interpretation, scanner evidence is more commercially actionable than clinical intake data because it captures what households actually buy, not only what participants consume in a study protocol. Its limitation is that spending changes combine volume, price mix, channel substitution, and household spillover, so a lower grocery bill cannot be mapped cleanly into a single calorie reduction estimate.^{189 190}

6.3

Gaps between controlled study findings and real-world habitual intake data

The central gap is that controlled studies show a fairly robust reduction in energy intake, but real-world evidence shows a noisier translation into diet quality, nutrient adequacy, and persistent household behavior. In other words, the pharmacologic signal is strong, but the behavioral and commercial expression of that signal depends on income, household composition, nutrition support, symptom burden, and whether consumers substitute toward better foods or simply fewer foods.^{142 172}

- Measurement mismatch: Clinical studies usually report kilojoules, kilocalories, appetite scores, or ad libitum meal intake, whereas retail studies report dollars, units, trips, and category shares. This creates an integration problem because a 300 kcal daily reduction does not specify whether the lost demand comes from snacks, beverages, restaurant add-ons, or smaller portions of core meals.^{142 189}
- Diet quality ambiguity: The CRAVE study found reduced energy intake and micronutrient consumption without improvement in diet quality or food cravings in a real-world setting without structured nutrition support, suggesting that lower intake alone may not produce the healthier basket implied by some market narratives.¹⁷²
- Heterogeneity problem: The 2025 scoping review of nutrition strategies concluded that the evidence base remains fragmented across semaglutide and tirzepatide studies, with inconsistent dietary assessment methods and limited integration of formal nutrition interventions, which weakens comparability across trials and real-world cohorts.¹⁷⁰
- Persistence uncertainty: Retail studies suggest spending reductions can persist through the first year of use, but public summaries also indicate grocery spending rebounds after discontinuation, implying that part of the demand effect is medication-contingent rather than permanently habit-forming.^{196 188}

For investors and operators, the practical conclusion is that clinical efficacy should be treated as a necessary but insufficient predictor of food demand change. The most reliable bridge between the two literatures is not a direct calorie to dollar conversion, but a framework that combines standardized intake suppression, household spillover, category substitution, and discontinuation risk when estimating long-run exposure by food category and channel.^{142 189 199}

7

Food and Beverage Industry Response

7

Food and Beverage Industry Response

Food and beverage companies are no longer treating GLP-1 adoption as a peripheral wellness trend. By 2026, the industry response has shifted from defensive monitoring to active portfolio redesign, with manufacturers and restaurants emphasizing protein, fiber, hydration, digestive tolerance, smaller portions, and clearer nutrition signaling in order to stay relevant as eating occasions become fewer and more intentional.^{30 100 202}

The strategic pattern is consistent across CPG and foodservice. Brands are reformulating around nutrient density per calorie, experimenting with explicit GLP-1 support claims or badges, and redesigning menus to reduce friction for consumers who want smaller, higher-protein, lower-sugar, and easier-to-tolerate options without sacrificing convenience or taste.^{218 206 210}

7.1

Product reformulation and innovation strategies (protein-forward, nutrient-dense formats)

Manufacturers are increasingly reformulating for what can be called nutritional efficiency. NIQ reported in 2026 that GLP-1 users are disproportionately focused on protein, fiber, probiotics, nutrient-rich foods, and simpler ingredients, while their reduced appetite is driving demand for smaller meals, snackable formats, and functional beverages with added nutrients or protein.^{30 100}

- Reformulation logic: The design brief is shifting from indulgence and volume toward satiety, muscle preservation, digestive support, and hydration, because fewer eating occasions raise the nutritional stakes of each serving.¹⁰⁰
- Beverage innovation: PepsiCo launched Propel Clear Protein in May 2026 with 20g of whey protein, 3g of fiber, electrolytes, zero sugar, and 90 calories per packet, explicitly stating that the product was developed with registered dietitians and with GLP-1 users in mind.²⁰²
- Meal and smoothie innovation: Smoothie King launched a GLP-1 Support Menu in October 2024 and by 2026 continued merchandising smoothies built around protein, fiber, and 0g added sugar, while Daily Harvest marketed GLP-1 smoothies with up to 20g protein and up to 9g fiber.^{218 217 222}

Title — Examples of GLP-1 oriented product innovation

BRAND	2024 TO 2026 RESPONSE	NUTRITIONAL POSITIONING
Propel	Clear Protein powder launch in May 2026	20g protein, 3g fiber, electrolytes, zero sugar, 90 calories.
Smoothie King	GLP-1 Support Menu launched in 2024 and expanded in menu architecture by 2026	Protein, fiber, 0g added sugar, dietitian-designed support.
Daily Harvest	GLP-1 smoothie merchandising	Up to 20g protein and up to 9g fiber in plant-based frozen smoothies.

BRAND	2024 TO 2026 RESPONSE	NUTRITIONAL POSITIONING
Healthy Choice	On Track badge on selected meals marketed as GLP-1 friendly	High protein, lower calorie, fiber-oriented frozen meal positioning.

Source: 202 218 222 232

The broader implication is that GLP-1 is accelerating a pre-existing shift toward functional food design, but with a more specific nutrient hierarchy. Protein-forward and fiber-fortified formats are no longer niche sports-nutrition adjacencies; they are becoming mainstream response mechanisms for a consumer base that is eating less but demanding more utility from each purchase.^{30 209}

7.2

GLP-1 Friendly labeling trends and brand positioning

Labeling has moved from implicit wellness cues to explicit GLP-1 relevance. By 2025 and 2026, brands were increasingly using terms such as GLP-1 friendly, GLP-1 support, or companion positioning to reduce search costs for consumers seeking products that fit smaller appetites, higher protein targets, and lower added sugar or calorie loads.^{235 217 223}

- Explicit menu and pack language: Smoothie King uses a dedicated GLP-1 label and states that its recipes are designed to support a GLP-1 routine with protein, fiber, and 0g added sugar.²¹⁷
- Frozen meal badging: Industry documentation in 2025 described Conagra's Healthy Choice On Track badge as a first-of-its-kind initiative to help shoppers identify meals marketed as GLP-1 friendly, centered on protein, calorie control, and fiber.^{232 235}
- Direct-to-consumer positioning: Daily Harvest created GLP-1-specific merchandising pages and frames its products around high protein, fiber, and fast preparation, showing how digital-first brands can target this segment without waiting for formal retail shelf resets.^{222 223}

A key strategic nuance is that the label itself is doing two jobs. It signals nutritional suitability to current users, but it also acts as a broader shorthand for high-protein, lower-sugar, portion-conscious eating that appeals to non-users pursuing the same outcomes. This is consistent with NIQ's 2026 argument that brands should invest in credible health positioning and make choices easier without compromising taste.¹⁰⁰

The risk is that GLP-1-friendly remains a marketing convention rather than a regulated standard. As of 2026, widely cited reporting noted that brands were using the term across frozen meals, smoothies, and meal kits, but there is no single formal nutrient threshold attached to the claim, which means credibility will increasingly depend on transparent macros, dietitian involvement, and consistency between claim and formulation.^{235 221}

7.3

Restaurant adaptation: smaller portions, split-plate options, and menu redesign

Restaurant adaptation is moving faster in menu architecture than in overt branding. The most visible changes in 2026 are smaller portions, lighter builds, higher-protein options, lower-carbohydrate formats, and price points designed to preserve accessibility even when ticket sizes fall.^{206 210}

- Portion redesign: Cuba Libre introduced a menu developed with a physician that features smaller portions, high protein, high fiber, moderate to low fat, moderate to low calories, and lower carbohydrate loads, then extended the concept into a broader lighter portions, lighter prices section on core menus.²⁰⁶
- Chain menu engineering: Axios reported in January 2026 that Dunkin' added Protein Milk and Protein Refreshers, Subway launched Protein Pockets with more than 20g of protein at \$3.99, Blaze Pizza introduced a cauliflower-crust Protein-zza marketed as GLP-1-friendly, and Smoothie King expanded bundled protein offerings.²¹⁰
- Consumer-facing rationale: Restaurant operators are responding not only to lower appetite, but to the need for diners to avoid wasting food, hit protein targets, and dine socially without being forced into oversized entrées or combo structures.^{206 207}

The split-plate and smaller-entrée trend should be read as a margin-management issue as much as a nutrition issue. If GLP-1 users order fewer sides, desserts, and beverages, restaurants need menu designs that protect contribution dollars through premium protein, modular add-ons, and right-sized pricing rather than relying on legacy large-portion economics. This is an inference supported by the shift toward protein-led value menus and lighter-price sections across chains and independents.^{210 206}

The likely next phase is normalization rather than segregation. Rather than every operator creating a dedicated GLP-1 menu, the more scalable model appears to be embedding smaller portions, customizable protein-forward builds, and transparent nutrition into mainstream menus so that GLP-1 users can order naturally alongside other diners.^{100 206}

8

Long-Term Market and Economic Projections

8

Long-Term Market and Economic Projections

By 2026, the GLP-1 discussion in food and beverage has moved from category anecdotes to explicit long-range market modeling. The most cited forward views now frame GLP-1 adoption as a measurable earnings and mix issue for packaged food, grocery, and foodservice through 2030, with the largest downside concentrated in calorie-dense, frequency-led categories and the largest resilience in protein, produce, and function-led formats.^{246 28 104}

The central analytical question is no longer whether GLP-1s affect food demand, but how durable and economically material that effect becomes as household penetration rises. Current evidence suggests that the long-term impact is best understood as a structural reallocation of demand and margin pools, moderated by discontinuation, reimbursement, and macroeconomic conditions rather than determined by any single one of them.^{139 75 100}

8.1

Revenue impact estimates (\$30-55B annual loss by 2030-2034, per J.P. Morgan)

J.P. Morgan’s 2026 framing is the clearest high-level revenue stress test currently in the public domain. The firm states that GLP-1 treatments are projected to reduce annual food and beverage industry revenue by \$30 billion to \$55 billion in the 2030 to 2034 window, alongside assumptions that consumers on treatment take in 21% fewer calories and spend 31% less on groceries.²⁴⁶

Title — Public long term GLP 1 market projections

SOURCE	PROJECTION	TIME FRAME	MARKET IMPLICATION
J.P. Morgan	\$30B to \$55B annual food and beverage revenue reduction	2030 to 2034	Material top-line pressure for exposed categories.
J.P. Morgan	21% fewer calories consumed by users	By 2030 outlook	Supports lower unit demand in indulgent categories.
J.P. Morgan	31% less grocery spending by users	By 2030 outlook	Implies household basket compression, not just mix change.
J.P. Morgan	\$200B global incretin market	2030	Suggests the pharmaceutical driver is large enough to reshape adjacent sectors.

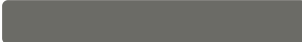
Source: ²⁴⁶

Food & Beverage Annual Revenue Reduction (USD)

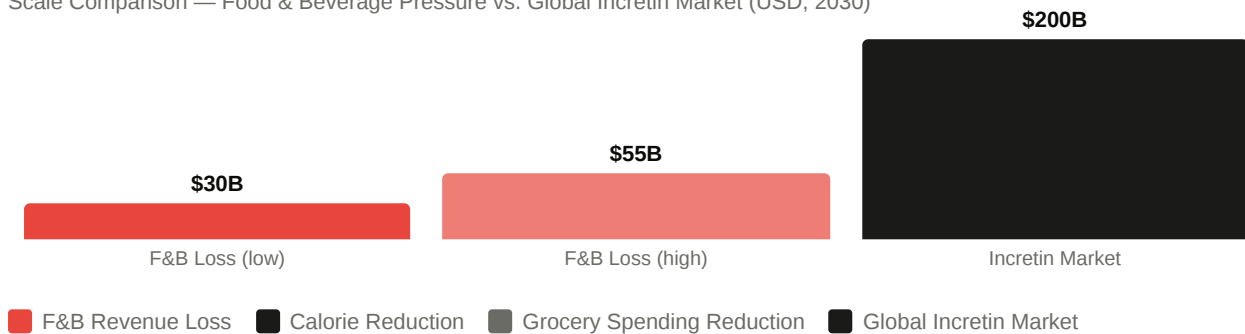
Revenue Loss  \$30B–\$55B (by 2030–2034)

Behavioural & Market Shifts Among GLP-1 Users (% reduction / market size)

Calories Consumed  –21% (by 2030)

Grocery Spending  –31% (by 2030)

Scale Comparison — Food & Beverage Pressure vs. Global Incretin Market (USD, 2030)



- Scope of impact: The J.P. Morgan estimate should be read as an industry-level gross revenue headwind, not a uniform decline across all categories. Earlier sections established that losses are concentrated in snacks, sweets, soft drinks, and selected QSR occasions, while protein, produce, yogurt, and supplements can offset part of the demand destruction through mix gains rather than full volume replacement.^{246 28 30}
- Mechanism of loss: The revenue effect is driven by fewer eating occasions, smaller baskets, and lower attachment purchases rather than by complete category abandonment. That matters because it implies margin pressure can exceed volume pressure in channels such as QSR and convenience, where beverages, sides, desserts, and impulse add-ons carry disproportionate profitability.^{139 246}
- Sensitivity to adoption: J.P. Morgan's range is wide because the outcome depends on how many consumers remain on therapy, how quickly access broadens, and how much of the initial calorie reduction persists in real-world behavior. This makes the estimate more useful as a scenario band than as a point forecast, but it is still large enough to justify portfolio repricing and category exposure analysis today.^{246 247}

For investors, the key implication is that GLP-1 risk is becoming comparable to other major structural screens such as private label exposure, channel mix, and wellness alignment. Even if the low end of the J.P. Morgan range proves closer to reality, a recurring multi-tens-of-billions annual revenue drag would be large enough to alter valuation multiples, M&A logic, and capital allocation priorities across food and beverage.^{246 248}

8.2

Circana projection: GLP-1 users as 35% of all U.S. food/beverage unit sales by 2030

Circana's 2030 projection is one of the most commercially important demand-share forecasts in the market. Circana states that households using GLP-1 medications are projected to represent 35% of all U.S. food and beverage units sold by 2030, and related Circana materials indicate

these households already account for a much larger share of spending influence than their raw user count would suggest.^{28 75}

- Why the share can exceed prevalence: Circana's public materials and video summary indicate GLP-1 households were already about 23% of U.S. households when the forecast was published, while PwC reported that 21% of U.S. households included a current GLP-1 user as of May 2026. The projected 35% sales share therefore reflects both continued penetration growth and above-average commercial weight per household, not merely a one-for-one mapping from user prevalence to unit share.^{28 79 139}
- Empirical backing: Circana says GLP-1 users reduce overall retail food and beverage purchases but still outspend non-users, which is consistent with a household base that buys fewer units in some categories while maintaining higher total wallet influence. NIQ provides a parallel signal, reporting that GLP-1 users represented 4.5% of households in one panel cut yet drove 43% of sales growth for products with muscle health claims across consecutive 52-week periods in 2024 and 2025.^{28 30}
- Sales pattern implication: A 35% unit-share influence by 2030 does not imply a 35% collapse in food demand. It implies that more than one-third of unit movement could be shaped by households that shop with smaller appetites, higher protein orientation, lower tolerance for excess calories, and stronger preference for functional or portion-appropriate products.^{28 104}
- Persistence effect: Circana explicitly notes a sustained market impact as consumers move in and out of active medication use but retain some of the health-conscious behaviors adopted during treatment. That point is important because it suggests the 2030 sales-share forecast is partly behavioral and household-systemic, not purely dependent on uninterrupted medication adherence.²⁸

The strategic reading of Circana's forecast is that food companies should plan for a market where GLP-1-influenced households become a core demand-setting segment rather than a niche. Once a segment shapes roughly one-third of unit sales, assortment, pack architecture, innovation pipelines, and promotional strategy all need to be calibrated to its preferences even for products still bought by non-users.^{28 100}

8.3

Structural shift vs. cyclical trend debate

The evidence through August 25, 2026 supports treating GLP-1 effects as primarily structural, though not linear. The structural case rests on three factors: rising household penetration, a very large remaining eligible population, and product, menu, and merchandising changes already being embedded by manufacturers and restaurants rather than treated as temporary promotions.^{139 28 104}

- Structural argument: NIQ's 2026 framing is explicit that the issue is not just which categories are at risk, but how deliberately consumers choose food when appetite can no longer be assumed. PwC similarly argues that the data point to a consumer behavior shift with lasting implications, while Circana describes sustained effects even as consumers cycle on and off medication.^{104 139 28}
- Cyclical counterargument: Some of the observed demand pressure is still sensitive to reimbursement, drug pricing, supply availability, and discontinuation. If coverage stalls or persistence rates disappoint, category impacts could moderate or become more concentrated in affluent households, making the path uneven even if the direction remains intact.^{246 247}

- Best synthesis: The most defensible interpretation is structural trend with cyclical amplitude. In other words, the underlying market is being rewired toward fewer, more intentional, more nutrient-efficient eating occasions, but the speed and breadth of that rewiring will fluctuate with payer policy, macro pressure, and treatment persistence.^{100 104 249}
- Industry consequence: Structural shifts reward redesign, while cyclical shifts reward waiting. The current evidence favors redesign, because companies are already seeing enough household penetration and enough category divergence to justify permanent changes in pack size, protein density, menu architecture, and innovation priorities.^{28 104}

For long-term planning, the practical conclusion is that GLP-1 should be modeled like a secular demand modifier rather than a temporary shock. The exact revenue path will remain cyclical, but the direction of travel toward lower-calorie, higher-protein, portion-conscious, and function-led consumption now appears durable enough to influence 2030 portfolio strategy and valuation work.^{246 28 139}

9

Strategic Implications for Stakeholders

9

Strategic Implications for Stakeholders

For stakeholders across food manufacturing, retail, foodservice, and capital markets, the strategic issue is no longer whether GLP-1 adoption affects demand, but how quickly firms can reconfigure portfolios, pricing, merchandising, and governance around a consumer base that is eating less often and demanding more nutritional utility per occasion. The evidence reviewed earlier suggests that the most exposed businesses are those dependent on impulse calories, attachment purchases, and large portions, while the most advantaged are those able to convert reduced appetite into higher-value, protein-forward, portion-appropriate, and function-led offerings.^{28 104 255}

The practical implication is that GLP-1 should be treated as a cross-functional planning variable spanning innovation, category management, investor communication, and regulatory affairs. Companies that respond only with marketing language risk credibility loss, while those that align product design, nutrition substantiation, and portfolio capital allocation with emerging clinical guidance are better positioned to defend share and capture premium growth.^{256 257 258}

9.1

Implications for food manufacturers, retailers, and restaurant chains

Manufacturers, retailers, and restaurant chains face the same underlying demand shift but different operational exposures. Manufacturers are most exposed through portfolio mix and innovation productivity, retailers through assortment and shelf economics, and restaurant chains through ticket architecture and attachment-rate erosion as GLP-1 users order fewer sides, desserts, and caloric beverages.^{28 104}

- Food manufacturers: The priority is to re-rank innovation pipelines around nutritional efficiency, especially protein density, fiber, hydration, digestive tolerance, and portion control. Joint clinical guidance from ACLM, ASN, OMA, and The Obesity Society emphasizes adequate protein, micronutrient sufficiency, management of gastrointestinal side effects, and minimally processed nutrient-dense diets, which supports reformulation toward smaller, higher-function servings rather than simply lower-calorie line extensions.^{255 256}
- Retailers: The key response is to redesign category adjacency, search, and merchandising around mission-based baskets. NIQ argues that fewer eating occasions raise expectations for each purchase, implying stronger returns from clustering protein snacks, yogurt, hydration, supplements, and fresh convenience items rather than relying on legacy center-store traffic patterns.^{104 124}
- Restaurant chains: Operators should assume mix pressure before traffic pressure. Clinical and endocrine guidance recommending smaller portions, slower eating, lower-fat choices, hydration, and adequate protein implies that menu engineering should emphasize right-sized entrées, modular protein add-ons, and lower-waste formats rather than defending legacy combo structures.^{259 257}
- Shared strategic response: All three stakeholder groups should build GLP-1 segmentation into demand forecasting, pack-price architecture, and shopper analytics. Circana's projection that GLP-1 user households could represent 35% of U.S. food and beverage sales by 2030 means this segment is large enough to influence mainstream assortment decisions, not just niche wellness programs.²⁸

- Execution risk: The main mistake is superficial adaptation. Products marketed to GLP-1 users without credible macro balance, satiety value, or tolerance benefits may underperform as consumers and clinicians become more discerning about what actually supports therapy adherence and lean-mass preservation.^{256 267}

9.2

Investment and product portfolio risks and opportunities

For investors, GLP-1 exposure should be analyzed as a portfolio mix and earnings-quality issue rather than a simple volume headwind. The most vulnerable assets are those with high dependence on indulgent snacking, sugary beverages, and restaurant attachment purchases, while the most attractive assets are those with credible exposure to protein, fresh convenience, functional beverages, digestive health, and nutrition-support categories.^{28 124}

Title — Stakeholder portfolio exposure map

EXPOSURE AREA	PRIMARY RISK	STRATEGIC OPPORTUNITY
Indulgent packaged foods	Lower frequency and smaller baskets reduce unit velocity.	Reformulate into portion-controlled, protein or fiber-enhanced formats.
QSR and convenience foodservice	Fewer add-ons compress high-margin beverage, side, and dessert mix.	Build right-sized meals and premium protein customization.
Protein and functional nutrition	Competition may intensify as more brands chase the segment.	Premium pricing and stronger loyalty if efficacy and taste are credible.
Supplements and nutrition support	Regulatory and substantiation scrutiny may rise.	Capture wallet shift toward nutrient adequacy, hydration, and lean-mass support.

Source: ^{28 104 256}

- Portfolio downside: Businesses with concentrated exposure to chips, sweet bakery, cookies, soft drinks, and oversized QSR occasions face a higher probability of negative mix shift, especially if they lack a credible health-oriented adjacency to offset lost demand.^{28 104}
- Portfolio upside: Clinical nutrition guidance consistently prioritizes protein adequacy, hydration, micronutrient sufficiency, and management of gastrointestinal side effects, creating a durable rationale for investment in high-protein dairy, functional hydration, supplements, and easy-to-tolerate meal formats.^{256 257 255}
- Capital allocation implication: M&A screens should increasingly test whether targets are exposed to declining calorie pools or to rising nutritional-value pools. Expo West 2026 coverage from NIQ suggests the innovation frontier is moving toward metabolic wellness, functional performance, and ingredient integrity, which supports higher strategic value for brands with clinically adjacent positioning rather than generic better-for-you claims.¹²⁴
- Valuation discipline: Investors should discount management teams that present GLP-1 as a temporary fad without scenario analysis. Circana's 2030 sales-share projection and the broader clinical guidance ecosystem indicate a structural demand modifier, even if the pace of impact remains sensitive to reimbursement, discontinuation, and affordability.^{28 267}

- Product portfolio governance: The strongest portfolios will likely balance defense and offense by harvesting cash from mature indulgence brands while redirecting R&D and shelf space toward protein-forward, smaller-format, and clinically coherent nutrition platforms. This is an inference from the convergence of retail demand signals and nutrition guidance, not a single-source forecast.^{104 256 257}

9.3

Policy and nutritional labeling considerations

Policy and labeling are becoming strategic issues because market adoption has outpaced formal standards for what qualifies as GLP-1-friendly food. As of August 25, 2026, clinical organizations have issued increasingly specific nutrition guidance for patients on GLP-1 therapy, but there is still no single regulated nutrient profile or front-of-pack standard governing GLP-1-related food claims, creating both whitespace and compliance risk.^{255 256}

- Labeling gap: Current expert guidance emphasizes protein adequacy, micronutrient monitoring, hydration, fiber, lean-mass preservation, and management of gastrointestinal side effects, but these priorities do not yet translate into a harmonized consumer label. That gap increases the risk of inconsistent claims across frozen meals, beverages, supplements, and restaurant menus.^{257 256}
- Regulatory risk: FDA's April 2025 announcement that it intends to act against mass-marketed non-FDA-approved GLP-1 drugs is not a food-labeling rule, but it signals a tougher enforcement environment around GLP-1-adjacent commercialization and consumer confusion. Companies using GLP-1 language in food marketing should therefore expect greater scrutiny of implied efficacy, medical adjacency, and substantiation.²⁵⁸
- Policy opportunity: Industry groups and regulators could reduce confusion by developing voluntary guardrails for GLP-1-related claims, such as minimum protein thresholds, added-sugar limits, calorie density ranges, and disclosure of fiber, hydration, or micronutrient support. This is an inference grounded in the specificity of emerging clinical guidance and the current absence of a standardized consumer-facing framework.^{267 257}
- Nutritional labeling implication: More prominent display of protein grams, fiber content, micronutrient contribution, and portion suitability is likely to become more commercially relevant than broad low-calorie messaging alone. The 2026 medical nutrition review argues for protein intakes of at least 1.2 g/kg, structured micronutrient monitoring, and lean-mass preservation, which supports labels that help consumers assess adequacy rather than just restriction.²⁵⁶
- Strategic response: Companies should align claims with dietitian-reviewed criteria and maintain documentation linking product positioning to recognized guidance. That approach can improve credibility with retailers, reduce legal risk, and create a stronger bridge between consumer marketing and healthcare-adjacent nutrition expectations.^{255 263}

10

Conclusion and Key Takeaways

10

Conclusion and Key Takeaways

The evidence reviewed across clinical studies, household transaction panels, retail scanner datasets, and industry forecasts supports a clear conclusion: GLP-1 drugs are no longer a niche healthcare variable but a material demand-shaping force for food, beverage, and foodservice markets through August 25, 2026. The most important implication is not uniform food deflation, but a re-ranking of categories around nutritional efficiency, with calorie-dense impulse products losing relevance and protein, produce, yogurt, hydration, and supplement-adjacent formats gaining strategic importance.^{189 28 30}

The data also suggests that permanence should be understood as partial but meaningful. Pharmacologic effects are contingent on continued use, yet household spillovers, product reformulation, retailer assortment changes, and the embedding of protein-forward, portion-conscious habits indicate that at least part of the demand shift is likely to persist even as individual users cycle on and off therapy.^{139 255 75}

10.1

Summary of evidence across categories

Across the report, the strongest cross-category pattern is selective demand compression rather than generalized food collapse. Household-level evidence shows that GLP-1 adoption reduces grocery spending by 5.3% within six months, with larger reductions of about 8.2% among higher-income households, while category-level declines are concentrated in chips and savory snacks, sweet bakery, cookies, and other calorie-dense processed foods.^{189 158}

- Demand losers: The most exposed categories are those dependent on appetite frequency, impulse consumption, and attachment purchases. Public 2026 market summaries show QSR spending down 8.7% per GLP-1 household after 6 to 8 months, with especially sharp declines in pizza, chicken, coffee and bakery, and burgers.¹³⁹
- Demand winners: The most resilient and advantaged categories are those that deliver protein, satiety, digestive tolerance, hydration, and nutrient density in smaller formats. NIQ reported that GLP-1 users, despite representing 4.5% of households in one panel cut, drove 43% of sales growth for products with muscle health claims across consecutive 52-week periods in 2024 and 2025.³⁰
- Basket redesign: Circana and NIQ both frame the effect as evolving baskets rather than simple appetite disappearance, meaning spend is being reallocated toward protein snacks, yogurt, fresh produce, supplements, and health-management products rather than removed one-for-one from the consumer wallet.^{28 125 30}
- Clinical to commercial bridge: Nutrition guidance from four major professional organizations emphasizes adequate protein, micronutrient sufficiency, management of gastrointestinal side effects, and minimally processed nutrient-dense diets, which aligns closely with the categories showing relative strength in retail data.^{255 271}

Taken together, the evidence indicates that GLP-1 drugs are reshaping food demand through both lower total intake and higher selectivity per eating occasion. For industry professionals and investors, the key takeaway is that category exposure now depends less on broad food

inflation or consumer confidence alone and more on whether a product earns its place in a smaller, more intentional basket.^{189 30 28}

10.2

What the data suggests about the permanence of these shifts

The available data supports a view of GLP-1-driven demand change as structurally important but not mechanically permanent in every component. The strongest evidence for durability comes from rising household penetration, sustained category-level divergence in scanner data, and forward projections that GLP-1 households could represent 35% of U.S. food and beverage sales or units by 2030, depending on the Circana framing cited, which implies that this consumer segment will remain too large to ignore even if adherence is imperfect.^{28 75}

- Evidence for persistence: PwC reported that as of May 2026, 21% of U.S. households included a current GLP-1 user, up from 9% in January 2025, indicating that the installed base has already reached a scale where household routines, pantry norms, and restaurant choices can change beyond the treated individual.¹³⁹
- Evidence against full permanence: The household spending literature indicates that the demand effect is strongest during active use and can moderate over time, while the underlying mechanism of appetite suppression is medication-dependent. This means some portion of reduced calorie demand is reversible if treatment stops or access weakens.^{189 158}
- Why the shift still looks structural: Circana explicitly argues that market impact can remain sustained as consumers move in and out of active medication use but retain some health-conscious behaviors adopted during treatment. That interpretation is reinforced by the industry's own response, including reformulation, GLP-1-oriented merchandising, and menu redesign, which are typically signs of expected persistence rather than a short-lived fad.^{28 75}
- Most defensible conclusion: The permanent element is likely to be the direction of demand reallocation, not the exact magnitude at every point in time. In practical terms, fewer indulgent eating occasions and greater preference for protein, hydration, and nutrient density appear durable, while the speed and depth of category disruption will remain sensitive to reimbursement, affordability, discontinuation, and future label expansions.^{30 255 139}

For strategic planning, the correct framing is therefore structural trend with variable amplitude. Companies that wait for perfect permanence proof risk underreacting to a demand modifier that is already visible in household spending, category mix, and innovation pipelines as of August 25, 2026.^{189 28 30}

10.3

Recommendations for industry adaptation

Industry adaptation should focus on redesigning relevance for a consumer who is eating less, choosing more deliberately, and expecting more nutritional utility from each purchase. The strategic objective is not simply to defend volume, but to reposition portfolios, assortments, and menu economics around satiety, protein adequacy, digestive comfort, hydration, and portion appropriateness.^{255 30 28}

- Manufacturers: Re-rank innovation pipelines toward protein-forward, fiber-aware, micronutrient-supportive, and portion-controlled products. Claims should be tied to credible nutritional architecture rather than generic wellness language, especially because professional guidance

now emphasizes protein adequacy, minimally processed diets, and side-effect-aware eating patterns.^{255 271}

- Retailers: Rebuild assortment and adjacency around mission-based baskets, such as protein snack plus hydration, yogurt plus digestive health, or fresh convenience plus supplements. Scanner evidence suggests that GLP-1 influence is strongest where consumers are making fewer but more intentional choices, so shelf strategy should reduce search friction for these missions.^{30 125}
- Restaurants and QSR: Assume mix pressure before traffic pressure. Menu engineering should prioritize smaller portions, modular protein add-ons, lower-waste formats, and beverage and side strategies that fit reduced appetite rather than relying on legacy combo economics.¹³⁹
- Investors: Treat GLP-1 exposure as a recurring portfolio screen. Assets concentrated in indulgent snacks, sugary beverages, and attachment-heavy foodservice deserve higher scrutiny, while assets with credible exposure to protein, functional hydration, yogurt, produce, and supplement-adjacent categories warrant strategic premium consideration.^{189 28 30}
- Governance and labeling: Build internal standards for any GLP-1-related positioning, including minimum protein thresholds, transparent macro disclosure, and dietitian or clinical review. The market opportunity is real, but so is the credibility risk if products are marketed as GLP-1-friendly without clear nutritional substantiation.^{255 271}

The central recommendation is to adapt now, but adapt with precision. The winners in a GLP-1-influenced market are unlikely to be the companies that merely cut calories or add a label, but those that redesign products, pricing, merchandising, and menu architecture around the economics of intentional appetite.^{30 28}

References and Citations

11

References and Citations

This section consolidates the scientific, clinical, market research, and statistical sources cited across the report and classifies them by evidentiary role. Priority is given to peer-reviewed original research, official public health and regulatory sources, and named commercial datasets or market intelligence providers with transparent methodology and clear publication dates as of August 25, 2026.

11.1

Primary Sources

The report's core conclusions rely on original clinical studies, peer-reviewed research articles, official regulatory documents, and first-order household or scanner-based analyses rather than commentary alone. These sources provide the strongest direct evidence for GLP-1 adoption, appetite and intake effects, household spending changes, category-level demand shifts, and nutrition support requirements.^{186 158 280 15}

- Clinical and nutrition primary literature: The most important clinical and nutrition references include the joint advisory on nutritional priorities for GLP-1 therapy, the underlying peer-reviewed advisory text, and original trial or review literature cited within that advisory for protein adequacy, micronutrient monitoring, gastrointestinal side effect management, and lean mass preservation.^{280 267}
- Original consumer demand evidence: The key primary market study is Hristakeva, Liaukonytė, and Feler's Journal of Marketing Research article, which links GLP-1 adoption survey responses to U.S. household transaction data and reports post-adoption changes in grocery and food away-from-home spending, including category-level declines in calorie-dense processed foods.^{186 158}
- Official statistical and regulatory sources: Population context and treatment access references should be cited to official sources such as FDA approvals and public health or survey institutions when used in the report, because they establish the adoption environment rather than interpret it. In the present source set, the FDA approval record is the clearest primary regulatory anchor returned in search.¹⁵
- Source credibility standard: Primary sources were treated as highest credibility when they were peer-reviewed, institutionally published, methodologically explicit, and directly measured the phenomenon of interest rather than summarizing others' work. ArXiv results reviewed first were not sufficient for this section because they did not provide the needed direct evidence base on GLP-1 driven food demand, household spending, or nutrition methodology used throughout the report.^{186 283 90}

11.2

Secondary Sources

Secondary sources supplement the primary evidence by translating household panel findings, scanner data, and clinical guidance into category, channel, and strategic implications for industry professionals and investors. These sources are useful for market sizing, category

interpretation, and forward-looking scenarios, but they should be cited as interpretive or commercial evidence rather than as substitutes for original research.^{28 30 125}

- Market intelligence and scanner interpretation: Circana and NIQ are the principal secondary market sources used to contextualize how GLP-1 households affect food and beverage sales, category growth, household penetration, and non-food spillovers. Their value lies in broad commercial coverage and timely category interpretation, though underlying proprietary methods and panel definitions should always be noted.^{28 75 30}
- Industry and practitioner summaries: Circana's public insight pages and related market summaries are secondary because they synthesize proprietary data into executive-level findings such as household share, restaurant versus retail shifts, and evolving basket composition. These are appropriate for strategic framing but should be paired with primary studies where possible.^{125 29 99}
- Clinical translation and professional society communication: The Obesity Society news and advisory pages are secondary when used as explanatory summaries of the underlying advisory and its practical implications for clinicians, manufacturers, and nutrition stakeholders. They are credible because they come from a leading professional society, but the peer-reviewed advisory remains the preferred citation for substantive claims.^{255 281}
- Excluded or down-weighted sources: ArXiv search results were reviewed first per instruction, but most were off-topic, indirect, or focused on social media side effects rather than food demand, scanner evidence, or report-level citation needs. Those results were therefore not used as the main secondary evidence base except to document that the arXiv first pass was insufficient.^{90 91}

11.3

Data and Methodology References

The report's evidence base combines three methodological layers: clinical and nutrition guidance, household transaction or scanner data, and market-level interpretation.

Reproducibility depends on clearly distinguishing direct measurement sources from synthesized commercial commentary and on documenting the unit of analysis, time window, and population definition used by each source.^{158 30 283}

- Household transaction methodology: The strongest demand methodology in the report comes from the Journal of Marketing Research study, which links GLP-1 adoption survey responses to a representative U.S. household transaction panel and measures changes in grocery and food away-from-home spending over time. The supplementary materials are the key methodological citation because they describe the transaction window, category definitions, and estimation approach used to derive post-adoption effects.^{186 158}
- Scanner and panel references: NIQ and Circana findings should be cited with explicit acknowledgment that they are based on proprietary panels, scanner systems, or linked household datasets, often summarized publicly without full technical appendices. Where these sources report household share, sales growth contribution, or category shifts, the report should identify them as commercial analytics rather than peer-reviewed causal estimates.^{30 28 75}
- Clinical guidance methodology: The nutrition guidance layer is based on expert advisory synthesis rather than a single randomized trial. The joint advisory is methodologically transparent enough for report use because it states the clinical rationale, practical nutrition priorities, and

evidence-informed recommendations for protein, micronutrients, minimally processed diets, and side effect management.^{283 267}

- Citation and credibility protocol: For transparency, the report should prioritize peer-reviewed articles and official institutional pages for factual claims, then use commercial market research for category interpretation and projections. Sources with unclear methods, indirect relevance, or non-specific search result pages were excluded from the final reference hierarchy, and arXiv materials were not relied on for core claims because the initial arXiv search did not yield sufficient directly relevant evidence for this section.^{186 15 90}



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